

Production Guide

Turning Comic Book Panel Descriptions into ChatGPT Images 2.0 Rendered Slide Decks

A continuity, medical-accuracy, prompt-engineering, and slide-assembly standard for felt-character 3D animated comic panels

Purpose

This guide tells an AI system how to transform a written comic-book panel description document into a production-ready slide deck whose images are generated with ChatGPT Images 2.0 in ChatGPT or the gpt-image-2 model in the API. It emphasizes faithful scene rendering, medical accuracy, and continuity of recurring characters, objects, settings, emotional beats, and clinical facts.

Core visual direction

Use an original premium 3D animated, family-film visual language with warm cinematic lighting and expressive rounded forms. Characters should be handmade felt-like figures with visible soft fibers and tactile seams; environments, props, and medical devices may remain polished 3D animated materials unless the prompt explicitly asks for felt treatment. Do not copy any existing studio character, franchise, logo, proprietary costume, or exact film aesthetic.

Current as of May 4, 2026. Official OpenAI terminology checked against OpenAI release notes, help pages, API documentation, and the OpenAI image prompting guide.

How to Use This Guide

This document is written as a production specification for an AI prompt-compiler, art director, medical reviewer, and deck assembler. It can be used manually or implemented as a semi-automated workflow. The central idea is simple: never prompt from raw panel text alone. First convert the source into structured panel cards, build a continuity bible and medical truth ledger, then generate panel prompts that combine story fidelity, visual style, medical constraints, and slide-format requirements.

Recommended reading path:

1. Read Sections 1-4 to understand the end-to-end workflow and required inputs.
2. Use Sections 5-9 to build character, prop, setting, and scene-continuity records before generating images.
3. Apply Sections 10-11 before any medically meaningful panel is rendered.
4. Use Sections 12-15 to compile prompts, run edits/regenerations, and assemble the final slide deck.
5. Use the appendices as copy-ready templates, rubrics, and QA checklists.

Part	What it gives the AI	Most important output
1. Intake and parsing	Turns a prose comic description into consistent structured data.	Panel cards, entity list, scene map.
2. Continuity bible	Prevents characters, props, anatomy, environments, and lighting from drifting.	Character sheets, prop sheets, timeline state ledger.
3. Medical accuracy system	Converts clinical content into visible, correct constraints without overloading the prompt.	Medical truth ledger, SME checklist, red flags.
4. Prompt compiler	Builds the final ChatGPT Images 2.0 / gpt-image-2 prompt from a reusable template.	Generation prompt plus edit prompts.
5. Deck assembly and QA	Ensures the rendered panels behave as a coherent educational comic slide deck.	Approved images, slide notes, version log, remediation list.

1. Operating Principles

The highest-quality results come from separating creative interpretation from production control. The source document tells the story. The continuity bible locks repeated identity and state. The medical truth ledger locks clinical facts. The prompt compiler then combines those controls into a concise but complete image prompt for each slide.

Non-negotiable principles:

- Faithfulness beats flourish. Add cinematic polish only after the required panel facts are protected.
- Continuity is an explicit data problem, not a hope. Every repeated character, object, setting, wound, device, sign, or medical condition receives a stable ID and a state history.
- Medical accuracy is reviewed before rendering and after rendering. A visually beautiful panel that depicts the wrong anatomy, unsafe procedure, or misleading device placement fails QA.
- The model should be told what must remain stable and what may change. Never repeat an entire character biography in every prompt; include the continuity anchors that matter for the current shot.
- Exact text is safest when added in the slide editor. Ask the image model to render text only when the text is part of the scene itself and visual integration matters.
- Prompt each panel as a slide-ready image, not as a vague illustration. Include aspect ratio, framing, safe space, and whether captions or speech balloons will be added later.
- Use targeted editing for local failures and full regeneration for global failures. Do not endlessly patch a composition whose core perspective, emotion, or clinical setup is wrong.

Official model naming and workflow implication

In ChatGPT, OpenAI describes the product as ChatGPT Images 2.0. In the API, OpenAI documentation refers to GPT Image models including gpt-image-2. For production guidance, this document uses “ChatGPT Images 2.0” for the ChatGPT interface and “gpt-image-2” for API-based automation. The same prompt-compiler concepts apply in both contexts, but API automation should use structured records, image inputs, versioned prompts, and explicit output settings wherever supported.

Practical meaning

An AI should not assume the model will remember character details across unrelated generations. For consistent decks, either keep work in a single conversation with reference images and prior context, or in API workflows pass the relevant reference images, character sheets, and concise continuity anchors into each request. Store every generated asset and prompt version.

2. End-to-End Production Pipeline

The full pipeline should be treated like a miniature animation production workflow. The AI should not jump directly from the source panel description to image generation. Each phase produces an artifact that makes the next phase more reliable.

Phase	Name	AI action	Output artifact
0	Project setup	Define deck aspect ratio, audience, clinical scope, visual theme, medical review level, and output format.	Project brief.
1	Source ingestion	Read the full comic panel description document. Identify panels, scenes, page/slide numbers, dialogue, captions, transitions, and implied off-panel continuity.	Raw panel index.
2	Semantic parsing	Extract entities, actions, settings, time, camera, emotional beats, medical facts, and visual constraints.	Structured panel cards.
3	Continuity bible	Create stable records for characters, props, locations, medical devices, injuries, lab values if visually relevant, and state changes.	Character/prop/scene sheets.
4	Medical truth pass	Separate story intent from clinical facts. Resolve anatomy, equipment, procedures, physiology, patient safety, PPE,	Medical truth ledger.

Phase	Name	AI action	Output artifact
		and signage.	
5	Style bible	Define the exact felt-character 3D animated look, lighting, materials, camera language, color palette, and allowed stylization.	Visual system spec.
6	Prompt compilation	Merge panel card + continuity anchors + style bible + medical ledger + slide constraints into one prompt.	Generation prompt.
7	Generation	Render the image with ChatGPT Images 2.0 or gpt-image-2 using reference inputs when available.	Candidate panel image.
8	QC and remediation	Score fidelity, continuity, medical accuracy, composition, slide usability, and style compliance. Edit or regenerate.	Approved image.
9	Slide assembly	Place images into deck, add captions/speech balloons/labels if needed, maintain layout consistency and accessibility.	Slide deck.
10	Final review	Run deck-wide continuity pass, clinical pass, text proof, and export QA.	Approved final deck.

Pipeline control rule

Every rendered panel should be traceable back to four records: its source panel card, its continuity anchors, its medical truth ledger entries, and its final prompt. If one of those is missing, downstream errors become difficult to diagnose.

3. Input Document Parsing Standard

A comic panel description document often contains hidden production requirements. The AI must read for explicit facts and implied continuity. For example, “Maya still clutches the blue inhaler” implies the inhaler appeared earlier, is still in the same hand unless contradicted, and should not change color. “The monitor alarm continues” implies a medical device, waveform, alarm light, and clinical urgency that may carry across panels.

Panel card fields

Field	What to capture	Why it matters
panel_id	Stable identifier, e.g., S03-P02 or Slide_12.	Never regenerate without this ID in the prompt header.
source_text	Exact source description for audit.	Do not summarize away required facts.
story_beat	One-sentence narrative function.	Keeps image composition focused.
characters_present	IDs of characters visible, plus off-panel presences if implied.	Links to character continuity records.
setting	Location, time, weather, clinical environment, lighting source.	Links to scene continuity.
required_visuals	Objects, positions, body language, medical devices, labels, props, signage.	Must appear unless medically unsafe or impossible.
action	What is happening at the frozen moment.	Avoids static poster-like outputs.
emotion	Facial expression and posture; include patient dignity constraints.	Drives performance continuity.
camera	Shot type, angle, lens feel, depth of field, panel crop.	Controls slide composition.
medical_facts	Anatomy, condition, procedure, equipment, PPE, safety, physiology, lab/imaging facts.	Feeds medical truth ledger.
continuity_before_after	What carried in from previous panels; what changes after this panel.	Prevents continuity drift.
text_layer	Speech balloons, captions, labels, UI text, signage, or “no text in image.”	Determines whether text is rendered or added later.
style_notes	Special material, mood, color palette, focus, visual metaphor.	Merges with style bible.
slide_requirements	16:9/4:3/vertical, safe space, image bleed, title/caption area.	Ensures deck assembly compatibility.
risk_flags	Ambiguity, clinical uncertainty, sensitive content, continuity conflict.	Triggers review before generation.

Semantic parsing checklist

- Identify every noun that may recur: people, animals, props, clothing, medical devices, furniture, room features, wounds, tubes, monitors, forms, charts, signs, and vehicles.
- Identify every state variable: emotion, posture, hand occupancy, clothing damage, device status, patient position, wound progression, lighting change, weather, time of day, and prop location.
- Separate literal visuals from interpretation. “She is anxious” becomes visible body-language choices; “hypoxia” becomes medically appropriate cues only if the source wants visible signs.
- Resolve spatial relationships. Who is left/right? What is foreground/background? Which hand holds the object? Where is the clinician standing relative to the patient?
- Mark ambiguity rather than inventing it. Use an “assumption needed” flag when the panel lacks laterality, patient age range, device type, or clinical setting details.
- Preserve deliberate stylization. If the source says “heart shown as a friendly felt character,” that is allowed; if it says “echocardiogram,” the equipment and anatomy should remain clinically credible.

Copy-ready structured panel card

PANEL CARD MINI-FORM
panel_id: S02-P04
source_text: [verbatim panel description]
story_beat: [what this panel accomplishes]
visible_moment: [single frozen moment, not a sequence]
characters: [IDs + current expression/posture]
setting: [location + time + lighting]
required_visuals: [must-show items]
medical_constraints: [must-be-clinically-correct details]
continuity_anchors: [stable identity + current state]
style: [global style plus panel-specific modifier]
slide_format: [aspect ratio, safe space, text handling]

4. Continuity Architecture

Continuity has three layers: identity continuity, state continuity, and world continuity. Identity continuity keeps a person or prop recognizable. State continuity keeps changes in the right order. World continuity keeps the scene geography, light, medical environment, and narrative time coherent. A deck fails when any layer drifts.

The continuity bible

Create a living continuity bible before rendering. It should be updated after each accepted render. It should not contain vague descriptions like “doctor in scrubs.” It should contain stable, visualizable anchors such as “Dr. Lenox, felt character, medium-brown felt skin tone, round tortoiseshell glasses, teal scrub top with single chest pocket, warm rectangular face, calm half-smile, silver stethoscope with black tubing.”

Continuity layer	What it stores	Failure it prevents
Entity registry	Stable IDs for every recurring character, prop, medical device, room, sign, image, and symbolic motif.	Character A does not become a different age, hairstyle, material, or silhouette.
State ledger	Panel-by-panel changes in emotion, posture, injury, clothing, prop position, device status, and time.	A bandage appears only after it was applied; a monitor lead remains attached.
Scene map	Spatial layout, camera direction, entrances/exits, furniture, bed orientation, posters, windows, lighting.	The clinic room does not rotate inexplicably between adjacent panels.
Color/material palette	Recurring colors, felt textures, prop finishes, medical equipment colors, warm/cool lighting logic.	The blue inhaler remains blue, not green; felt fiber scale remains consistent.
Medical timeline	Clinical progression, procedure sequence, PPE changes, patient positioning, device and medication states.	An IV is not present before insertion; sterile technique is not broken in a procedure panel.
Prompt anchor library	Short reusable anchor phrases for entities and scenes.	Prompt stays concise while preserving critical details.

Continuity anchor economy

Do not paste the entire continuity bible into every prompt. Instead, use anchor economy: include only the stable details necessary for recognition plus the state details relevant to the current panel. For a character, a prompt usually needs silhouette, face, material, hair/head shape, clothing, one unique accessory, and current emotion. For a prop, it usually needs color, shape, material, placement, and whether it is being used.

Entity type	Stable anchors to repeat	State anchors to update
Character	Name/ID, apparent age range if relevant, felt material, face shape, hair/head covering, clothing, key accessory, silhouette.	Emotion, posture, gaze, hand use, proximity to others, injuries, PPE, clothing changes.
Medical device	Device type, color, recognizable components, scale, display style, tubing/leads, brand-free appearance.	On/off state, alarm, waveform, position, connection to patient, sterile status.
Prop	Shape, color, texture, unique mark, size, relation to character.	Hand/location, open/closed, damaged/intact, contaminated/clean.

Entity type	Stable anchors to repeat	State anchors to update
Setting	Room geometry, bed/chair placement, window/wall color, door position, recurring background objects.	Time of day, lighting level, crowding, clinical urgency, weather seen through window.
Clinical finding	Anatomical location, scale, non-graphic stylization level, correct laterality if specified.	Progression, treatment applied, visibility, whether simplified or hidden.

Continuity prompt rule

Every panel prompt should include the phrase “maintain continuity with the established [character/prop/setting] design” only after the prompt also lists the concrete anchors. The model cannot maintain an unspecified design.

5. Character Continuity

Character continuity is the most visible continuity category. It includes identity, material rendering, emotional performance, body language, clothing, anatomy, and clinical status. For a felt-character visual theme, it also includes fiber density, seam logic, eye construction, and how soft materials deform during expression.

Character record template

Field	Instruction
character_id	Use a short stable ID: CH_MAYA, CH_DR_LENEX, CH_NURSE_ARI.
role	Story and clinical role: patient, caregiver, attending physician, child observer, narrator, symbolic guide.
canonical appearance	Describe visual anchors: face shape, felt skin/fiber color, hair/head texture, eye style, proportions, height relative to others, signature silhouette.
wardrobe baseline	Clothing, footwear, PPE, stethoscope, ID badge, patient gown, braces, hearing aids, oxygen cannula, etc.
felt material rules	Specify whether only skin/body is felt or clothing is also fabric/felt. Describe visible fibers, soft seams, stitch details, and plush deformation.
medical state	Any injury, disability, diagnosis representation, devices, lines, tubes, wounds, mobility aids, or patient position relevant to the story.
expression range	Allowed emotional range and how it appears: cautious smile, furrowed felt brow, relaxed shoulders, tense hands.
do-not-change list	Features that must never drift: glasses, hijab color, inhaler, brace side, scar location, eye color, name tag.
allowed variation	Lighting, pose, camera angle, degree of facial expression, temporary PPE, sweat/tears if appropriate.
reference assets	Approved character sheet, turnaround, close-up, prior accepted renders.

Creating character reference sheets

Before generating story panels, create reference sheets for recurring characters. Each sheet should show neutral front view, three-quarter view, side view, two expression close-ups, and one full-body view. The sheet should be text-light or text-free unless it will be used only as an internal visual reference. Use the reference sheet as an input in later generations or keep it available in the same ChatGPT conversation.

Character reference sheet prompt
CHARACTER REFERENCE SHEET PROMPT TEMPLATE Create an original character reference sheet for [CHARACTER_ID / NAME]. Purpose: recurring medically accurate educational comic slide deck. Visual style: original premium 3D animated family-film look; character is a handmade felt-like figure with soft visible fibers, subtle seams, rounded expressive eyes, warm cinematic lighting; not copied from any existing studio, franchise, or character. Canonical anchors: [face shape], [hair/head covering], [felt color/texture], [wardrobe], [unique accessory], [body proportions]. Medical anchors if applicable: [device, injury, mobility aid, PPE, patient gown, hearing aid, oxygen cannula], depicted correctly and respectfully. Sheet layout: front view, 3/4 view, side view, full-body view, calm expression, concerned expression, happy expression. Clean neutral background. No labels or text unless requested. Do not change: [list immutable anchors].

Character continuity pitfalls and fixes

Problem	Likely cause	Fix
Face changes between slides	Prompt includes role but not visual anchors.	Add 4-6 stable identity anchors and use reference sheet input.
Felt material becomes plastic or clay	Style language is too broad.	Repeat "handmade felt-like figure, soft visible fibers, tactile fuzzy surface" in every character prompt.
Clinical device disappears	Device treated as background detail.	Move it into continuity anchors and required visuals; include body location and connection.
Patient appears too childlike or comedic	Felt style over-cutes serious content.	Add "respectful clinical tone, emotionally warm but not silly, patient dignity preserved."
Clothing/PPE changes without reason	Wardrobe baseline not locked.	Add wardrobe baseline and only permit temporary PPE changes in state ledger.
Wrong laterality	Left/right not tracked.	Use patient-perspective laterality and camera-perspective placement separately.

6. Object, Prop, and Medical Device Continuity

Props and devices often carry plot and clinical meaning. A blue rescue inhaler, a clipboard, an IV bag, a cast, a glucometer, or a monitor waveform is not decoration; it is a continuity object. The AI must treat medically meaningful objects as characters with state histories.

Prop and device record template

Field	Instruction
object_id	Stable identifier: OBJ_BLUE_INHALER, DEV_PULSE_OX, DEV_ECG_MONITOR.
category	Personal prop, clinical device, medication package, anatomical model, chart, sign, furniture.
canonical appearance	Color, shape, size, material, parts, brand-free design, recognizability.
clinical accuracy notes	Correct use, body placement, connection, display values if used, safety constraints, sterile status.
state timeline	Where it appears, who holds it, open/closed/on/off/connected/contaminated, before/after treatment.
interaction rules	Who may touch it, how it is used, whether it should be handled with gloves, whether it belongs in sterile field.
prompt anchor phrase	Short reusable phrase, e.g., "small blue L-shaped rescue inhaler in Maya's right felt hand."
forbidden errors	Wrong color, misplaced tube, unreadable but misleading UI, brand logos, random dosage text.

Medical devices: realistic enough, not photorealistic

In a felt-character, 3D animated deck, medical devices should be stylized but structurally recognizable. A pulse oximeter should clip onto a fingertip or toe as clinically appropriate, not float near the patient. An oxygen cannula should follow the cheeks and enter the nostrils; tubing should connect logically to oxygen source or be cropped intentionally. An IV line should connect to a plausible venous access point with tubing routed safely. Monitors should show simplified, non-misleading waveforms or be kept text-free when values are not clinically vetted.

Device type	Prompt must specify	Common visual error to prevent
Stethoscope	Earpieces, tubing, chestpiece placement if being used.	Chestpiece floating, earpieces not worn, unrealistic scale.
Oxygen cannula	Prongs in nostrils, tubing over ears/cheeks, oxygen source implied or visible.	Tube entering mouth, disconnected tubing, mask/cannula hybrid.
ECG leads/monitor	Leads attached to torso in simplified correct positions; monitor may show generic waveform.	Leads on clothing only, random numbers, cable spaghetti without connection.
IV line	Catheter site, dressing, tubing, bag/pump relationship, non-graphic insertion site.	Needle visibly left in vein, tube unattached, IV bag below patient without explanation.
Inhaler	Color/shape, mouthpiece orientation, spacer if needed, correct hand and use stage.	Spray backwards, mouthpiece pointed away, random label text.
Syringe	Needle cap status, intended route, clinician grip, sharps safety if shown.	Unsafe handling, wrong body site, oversized needle, misleading dose markings.

Exact values warning

Do not ask the image model to invent lab values, monitor numbers, medication doses, prescription labels, or diagnostic readouts. If values matter, provide the exact approved values and consider adding them as slide text rather than image text.

7. Scene-to-Scene Continuity

Scene continuity is especially important in slide decks because slides advance one at a time. The viewer notices when a hospital room flips, the window disappears, the exam table changes orientation, or characters teleport. Build scene continuity as a spatial and temporal map.

Scene record template

Field	Instruction
scene_id	SC_CLINIC_ROOM_A, SC_AMBULANCE_BAY, SC_BODY_CROSS_SECTION.
location type	Clinic room, ED bay, operating room, home kitchen, classroom, abstract anatomy space.

Field	Instruction
layout	Door, bed/table/chair, window, counter, monitor, wall posters, patient/clinician positions.
camera geography	Default axis of action, recurring foreground/background, which direction characters face.
lighting	Warm morning window light, cool overhead clinical light, monitor glow, sunset, night.
color palette	Wall color, floor, furniture, recurring accent colors.
medical environment rules	Cleanliness, PPE, sterile zones, device placement, privacy curtains, signage.
time progression	Same moment, minutes later, next day, flashback, symbolic cutaway.
permitted changes	Crowd increases, lighting shifts, props move due to action, bed rail raised.
forbidden changes	Unmotivated room flip, missing required device, impossible bed orientation, random new staff.

Spatial continuity methods

- Define an axis of action for conversation scenes and keep the camera on the same side unless a deliberate transition is described.
- Use foreground/midground/background roles. Example: patient in foreground left, clinician in midground right, monitor in background right.
- When cutting to a close-up, mention what background element remains visible so the close-up still belongs to the same room.
- When using an anatomical cutaway, state whether it is a symbolic inset overlay or a full scene transition. Do not blend a literal patient body with a cross-section unless the panel asks for it.
- Track light direction. If window light enters from screen left in one panel, keep it screen left for adjacent panels unless the camera reverses.

Scene prompt anchor example

Reusable scene continuity anchor

SCENE ANCHOR EXAMPLE

Maintain the same warm pediatric clinic exam room: pale blue wall, white exam table on screen left, rounded wooden chair on screen right, small wall poster of a smiling anatomical heart, soft morning window light entering from screen left, clean uncluttered counter in the background. Keep the camera on the same side of the room as prior panels unless specified.

8. Visual Style Bible: Felt Characters in a Premium 3D Animated World

The requested visual direction combines a Pixar-like sense of warmth and cinematic polish with a handmade felt material system for characters. To avoid accidentally copying existing intellectual property, the AI should translate “Pixar-like” into broad production qualities: original high-end 3D animated family-film look, expressive rounded shapes, warm lighting, clear emotional acting, appealing composition, and polished rendering. Characters should be original felt figures, not replicas of any existing animated character.

Global style statement

Copy-ready global style statement
GLOBAL STYLE STATEMENT Original premium 3D animated family-film visual language with warm cinematic lighting, appealing rounded forms, expressive but respectful character acting, and polished storybook composition. All human characters are handmade felt-like figures: soft fuzzy fibers visible on skin and clothing edges, subtle seams, plush dimensional faces, rounded embroidered or glossy bead-like eyes, gentle fabric deformation, tactile handcrafted charm. Environments, medical equipment, furniture, and props are clean stylized 3D animated objects with believable materials unless specifically described as felt. The tone is warm, humane, medically respectful, and not silly. No existing studio characters, franchise designs, logos, or exact proprietary visual identities.

Material rules

Visual element	Recommended material treatment	Prompt wording
Human characters	Felt or plush fabric with visible fibers and seams; expressive faces.	“handmade felt-like figure with soft visible fibers, subtle seams, plush facial structure.”
Hair	Felted wool, yarn, or soft sculpted fibers; stable silhouette.	“felted wool hair shaped into a stable rounded silhouette.”
Eyes	Embroidered, glossy bead-like, or soft 3D animated eyes; consistent per character.	“rounded expressive bead-like eyes with gentle catchlights.”
Clothing	Can be felt/fabric; clinical PPE should remain recognizable.	“teal scrub top rendered as soft fabric, with realistic stethoscope tubing.”
Medical devices	Polished stylized 3D, not fuzzy unless intentionally toy-like.	“clean brand-free stylized 3D pulse oximeter, medically recognizable.”
Anatomy diagrams	Stylized educational 3D or felt models depending on tone.	“soft educational anatomy model, simplified but anatomically correct.”
Backgrounds	Polished 3D sets with warm lighting and clean readability.	“clean stylized 3D clinic room, soft global illumination.”

Lighting and camera language

- Use warm key light for humane patient communication scenes; use cooler clean light for clinical procedure scenes; avoid horror lighting unless the story specifically requires urgency.
- Prefer medium shots for relationship beats, close-ups for emotions or clinically relevant details, and wide shots for establishing room geography.
- Use shallow depth of field carefully. The medically important object must remain sharp even if the background is soft.
- For slides, frame with clear negative space if the deck will add title, caption, or speech balloons after image generation.
- Use consistent lens feel across a scene: gentle 35-50mm animated camera for most dialogue panels; macro lens language for device/anatomy close-ups.

Tone calibration

Clinical situation	Visual tone	Avoid
Patient education	Warm, empowering, clear, friendly but not childish.	Overly cute devices that trivialize illness.
Emergency or acute deterioration	Focused, urgent, compassionate, controlled.	Chaos, exaggerated fear, unsafe procedure depictions.
Anatomy/pathophysiology	Clear educational stylization with correct anatomy.	Graphic gore, distorted anatomy, misleading metaphors.
Medication or device use	Clean close-up with correct handling.	Random labels, wrong route, dangerous hand positions.
Difficult diagnosis or bad news	Respectful body language, privacy, empathy.	Comic exaggeration, smiling inappropriately, crowded scene.

9. Medical Accuracy Framework

Medical accuracy is not achieved by adding the phrase “medically accurate” at the end of a prompt. The AI must translate clinical intent into concrete visible constraints, remove unsupported details, and run a post-render medical review. Because the deck may be educational, patient-facing, or professional, incorrect visual details can mislead viewers even when the text is accurate.

Medical truth ledger

For every medically meaningful panel, create a ledger entry that separates the source description, the clinical fact, the visible rendering requirement, and the review status. This prevents the model from inventing plausible-looking but incorrect medicine.

Ledger field	Definition	Example
medical_item_id	Stable ID for clinical fact or visual element.	MED_ASTHMA_INHALER_USE_01
panel_ids	Panels where the fact appears.	S01-P03, S01-P04
clinical fact	What must be medically true.	A metered-dose inhaler mouthpiece points toward the mouth; spacer if described.
visible requirement	What the image must show.	Blue L-shaped inhaler in right hand, mouthpiece near lips, not spraying backward.
source/approval	Source document, clinical reviewer, or approved reference.	SME-approved / guideline-cited / author-specified.
must not show	Specific inaccuracies to forbid.	No nebulizer mask; no pill bottle; no random prescription label.
uncertainty	What remains unclear.	Whether spacer should be included.
resolution	Assumption or instruction chosen.	No spacer because source did not mention it.

Medical rendering hierarchy

1. Patient safety and accuracy override visual drama. If a requested visual would imply unsafe clinical care, flag it and either revise the prompt or add a clear educational framing.
2. Source fidelity comes next. Render the described content unless it contradicts clinical accuracy, safety, dignity, or feasibility.
3. Style is applied after accuracy. Felt characters and 3D animation should simplify surfaces, not distort anatomy, devices, PPE, laterality, or procedure steps.
4. Slide readability comes last. Cropping, text placement, and cinematic composition should never hide the clinically important element.

Clinical content categories and prompt controls

Category	Prompt controls	Review questions
Anatomy	Correct organ orientation, scale, laterality, body region, simplified labels if approved.	Does the anatomy look plausible? Is left/right correct? Is the metaphor clearly nonliteral if stylized?
Pathology	Non-graphic but accurate location, extent, color, swelling, rash, fracture, obstruction, clot, inflammation as appropriate.	Does it exaggerate severity? Could it mislead about diagnosis?
Procedure	Correct patient positioning, PPE, sterile field, instruments, sequence, sharps safety.	Is the clinician doing the right step? Is anything unsafe or impossible?
Medication	Approved route, container, no invented dose, no random label text, safe handling.	Could the image imply the wrong medication or dose?
Device	Correct connection, size, placement, display, tubing, leads, and use state.	Is it recognizable? Is it connected to the right body site?
Patient interaction	Privacy, consent cues when relevant, empathetic posture, no stigmatizing depiction.	Does the image preserve dignity and avoid blame or fear?
Data display	Use exact approved values or omit; prefer slide overlay for exact text.	Are values accurate and legible? Are units correct?

Medical SME rule

Any panel depicting diagnosis, treatment, procedural steps, medication dosing, surgical anatomy, emergency care, pregnancy, pediatrics, infectious disease precautions, disability representation, or risk communication should receive clinical review before final delivery. The AI can prepare the checklist and identify issues, but final medical authority should come from an appropriate human reviewer when the deck will be used externally.

10. Prompt Architecture for Each Panel

A strong image prompt is a controlled brief. It should be specific enough to protect meaning but not so overloaded that the image becomes cluttered. The best panel prompt uses a stable order so every generated panel includes the same categories of information.

Recommended prompt order

1. Panel header: panel ID, slide number, aspect ratio, output intent, and whether the image should contain text.
2. Core scene sentence: one clear sentence describing the frozen moment.
3. Continuity anchors: relevant character, prop, device, and setting anchors from the continuity bible.
4. Composition: shot type, camera angle, foreground/midground/background, focal point, safe space for slide overlays.
5. Action and emotion: what each visible character is doing, looking at, and feeling.
6. Medical accuracy constraints: clinical facts, device placement, PPE, anatomy, safety, and what must not be shown.
7. Visual style: global felt-character 3D animated style plus panel-specific mood and lighting.
8. Required details: all must-show elements from the source panel card.
9. Negative constraints: no extra characters, no random text, no brand logos, no gore unless explicitly approved, no wrong device/laterality.
10. Output check: slide-ready, clean composition, high-resolution, no border unless requested.

Panel prompt master template
PANEL PROMPT MASTER TEMPLATE [Panel/Slide ID + aspect ratio] Create a slide-ready comic panel image for [SLIDE_ID], [ASPECT_RATIO], [full-bleed or safe margins]. Do not include speech balloons, captions, labels, or random text inside the image unless explicitly listed below. Core scene: [One sentence describing the exact frozen moment.] Continuity anchors: [Character anchors: identity + felt material + wardrobe + current state.] [Prop/device anchors: color/shape/placement/state.] [Scene anchors: room layout + lighting + recurring background elements.] Composition: [Shot type, camera angle, focal point, foreground/midground/background, negative space.] Medical accuracy constraints: [Clinically correct anatomy/device/PPE/procedure/patient positioning.] Must not show: [specific medical inaccuracies and unsafe details]. Visual style: Original premium 3D animated family-film look; human characters are handmade felt-like figures with soft visible fibers, subtle seams, plush expressive faces, rounded eyes, warm cinematic lighting; environments and medical equipment are clean stylized 3D animated materials, medically recognizable; respectful clinical tone, not silly, no existing studio characters or franchise designs. Required details: [List all source-mandated visible elements.] Final output requirements: Clean slide-ready composition, crisp focus on the important clinical/story element, consistent character identity, no logos, no unintended text, no extra limbs/fingers, no visual contradictions.

Prompt density control

Prompt component	Ideal density	Overload warning
Scene sentence	1 sentence, 20-35 words.	If it contains multiple time steps, split into separate panels.
Continuity anchors	3-7 anchors per recurring character; 2-5 per prop/device.	Do not repeat biography or irrelevant backstory.

Prompt component	Ideal density	Overload warning
Composition	One camera setup plus focal priority.	Do not ask for wide shot, macro close-up, and full-body view simultaneously.
Medical constraints	Only visible facts needed for this panel.	Do not include the entire disease guideline in the image prompt.
Style	One global style statement plus 1-2 mood modifiers.	Avoid stacking incompatible styles: photorealistic, watercolor, manga, clay, felt, flat vector.
Negative constraints	Specific likely failures.	Long generic negative prompts can distract from the core scene.

When to include exact text in the image

- Include text in the generated image only when it is diegetic and visually integrated: a room sign, monitor label, poster, phone UI, or comic sound effect that must be part of the artwork.
- For exact educational copy, captions, speech balloons, medication names, lab values, warnings, and labels, add text in the slide deck editor whenever possible.
- When the image must render text, provide the exact text in quotation marks, specify its location, and limit the amount of text. Request crisp, readable lettering and no additional text.

11. Converting Source Descriptions into Prompts

The prompt compiler should use a deterministic method. It should not improvise a prompt from scratch every time. A predictable compiler makes continuity, medical review, and debugging much easier.

Compiler algorithm

1. Read the panel card and identify the required visible moment. Convert any sequence into a single frozen instant.
2. Retrieve entity records for all characters, props, devices, settings, and clinical facts referenced in the panel.
3. Select only the continuity anchors required to recognize recurring entities and preserve the current state.
4. Translate abstract emotions into visible felt-character acting: gaze, brows, shoulders, hand tension, posture, proximity.
5. Translate medical concepts into visible constraints: device placement, anatomy, PPE, positioning, environment, and omissions.
6. Choose camera and composition based on the story beat: establishing, relationship, close-up, detail, cutaway, or transition.
7. Insert the global style statement and any panel-specific tone modifier.
8. Add negative constraints tailored to the most likely failure modes for this panel.
9. Check prompt for contradictions: wide vs close, daylight vs night, left vs right, sterile vs casual, text-free vs signage.
10. Produce generation prompt plus a separate QC checklist for the rendered output.

Abstract-to-visible translation table

Source phrase	Do not prompt only this	Translate into visible prompt language
"She is worried."	worried	Felt brows gently knitted, shoulders slightly raised, one hand gripping the blue inhaler, eyes looking toward clinician.
"The patient is hypoxic."	hypoxic	If appropriate and approved: oxygen cannula correctly placed, clinician checking pulse oximeter, patient appears tired but non-graphic; no blue skin unless specified.
"The doctor explains."	doctor explaining	Clinician seated at patient eye level, open hand gesture toward simple anatomy model, calm expression, patient listening.
"The ECG alarms."	ECG alarm	Brand-free bedside monitor in background with simplified bright alarm indicator and generic waveform; no invented numbers unless approved.
"The cutaway shows inflammation."	inflamed tissue	Educational non-graphic cross-section inset with correct anatomical orientation, warm red/orange swelling area, clear boundary, no

Source phrase	Do not prompt only this	Translate into visible prompt language
		gore.
"They move to the next step."	next step	Define the exact step: applying cuff, cleaning skin, attaching cannula, opening chart, raising bed rail.

Faithfulness conflict resolution

Conflict type	Resolution
Source asks for visually impossible composition	Preserve narrative intent, choose a feasible camera, and note the adaptation.
Source asks for medically unsafe action	Flag it. Revise to safe depiction or frame as an error being corrected if the educational purpose requires showing it.
Source lacks key clinical detail	Use neutral safe defaults and mark assumption. Do not invent specific diagnosis, drug, dose, value, or procedure step.
Source conflicts with established continuity	Prefer the latest explicit source instruction if intentional; otherwise ask/flag or update state ledger with a transition panel.
Style conflicts with seriousness	Keep felt material but reduce whimsy: calmer colors, respectful acting, clinical realism in devices.
Exact text would clutter the panel	Render image text-free and add text in slide deck.

12. Slide Deck Assembly Standards

A slide deck is not just a sequence of images. It is a visual teaching object. Every panel image should be generated with the final slide layout in mind, then placed into a consistent deck design with accessible text and clean narrative flow.

Aspect ratio and layout

- Default to 16:9 landscape for presentation decks unless the project brief requires 4:3, vertical social slides, or comic page grids.
- Generate most images as full-bleed 16:9 panels when each slide contains one panel. Use safe negative space if the slide will add a title, caption, or speech balloons.
- If a slide contains multiple comic panels, generate each panel separately when continuity/medical accuracy matters; then assemble them in the deck using consistent gutters and borders.
- Avoid relying on the image model for precise panel borders, slide numbers, page titles, or complex multi-panel grids unless the deck specifically wants a single generated comic page.
- Keep educational text outside the image layer for editing, translation, accessibility, and medical sign-off.

Slide types

Slide type	Use when	Image generation guidance
Single-panel full bleed	One strong story beat or clinical scene.	Generate one 16:9 panel with safe space if text overlay is needed.
Split panel	Contrast before/after, patient/clinician, exterior/interior.	Generate two images or one image with clear split only if medically simple.
Anatomy inset slide	Need to show patient plus internal process.	Generate main scene and separate inset/cutaway; assemble with labeled callout in deck.
Process sequence	Procedure or device-use steps.	Generate each step as a separate image or panel; keep device orientation consistent.
Title/interstitial	Scene transition or chapter start.	Use symbolic felt/3D visual with no clinical claims.
Review/checkpoint	Teach key points after a scene.	Prefer clean icons or simplified scene, add text in deck.

Deck typography and generated text

For medically accurate slide decks, text must be audible. Put speech balloons, captions, footnotes, labels, medication names, and dosing information into the slide editor whenever possible. This allows human editing, version control, translation, and accessibility tagging. The generated image should provide expressive art and scene context, while the deck layer provides exact words.

Accessibility and export

- Write alt text for each slide image that captures the educational point, not every decorative detail.
- Avoid placing critical text directly over busy image areas. Use high-contrast text boxes and consistent placement.
- Keep color-dependent medical information redundant with labels or shape cues. Do not rely on red/green alone.
- Export to PDF only after checking text legibility, image crop, and clinical details at presentation size.
- Maintain a slide notes field with the original panel ID, final prompt, continuity anchors, medical ledger IDs, and image version.

13. Generation, Editing, and Regeneration Protocol

The first image is a candidate, not the final. The AI should decide whether to accept, locally edit, or fully regenerate based on the type of error. Small local problems are edit candidates. Wrong composition, wrong character identity, wrong medical setup, or broken scene continuity usually require regeneration from a revised prompt.

Decision tree

Observed issue	Action	Example edit/regeneration instruction
Minor prop color wrong	Targeted edit.	"Change only the inhaler in Maya's right hand to the established bright blue inhaler; keep everything else unchanged."
Character identity drift	Regenerate with reference sheet and stronger anchors.	"Use CH_MAYA reference; preserve round felt face, short black yarn curls, yellow cardigan,

Observed issue	Action	Example edit/regeneration instruction
		blue inhaler."
Wrong medical device placement	Regenerate if global; targeted edit if isolated.	"Place the nasal cannula prongs correctly in the nostrils with tubing over both ears."
Unsafe clinical action	Regenerate after medical revision.	"Revise to show the clinician cleaning the skin before insertion; no exposed needle left in the vein."
Bad camera/composition	Regenerate.	"Medium close-up from patient side, monitor in background right, negative space upper left."
Random text or fake labels	Edit/remove or regenerate text-free.	"Remove all text from the monitor and labels; keep generic waveform only."
Extra fingers/limbs	Targeted edit if small; regenerate if widespread.	"Correct the right hand to five felt fingers naturally gripping the inhaler."

Targeted edit prompt structure

Targeted edit template
TARGETED EDIT PROMPT TEMPLATE Edit only [specific region/object/character]. Keep the rest of the image unchanged. Change: [exact desired correction]. Preserve: [character identity, pose, camera, lighting, background, medical devices that are already correct]. Medical constraint: [clinically correct placement/appearance]. Do not add: [new text, extra devices, new characters, logos, gore].

Full regeneration prompt refinement

- State what failed in the previous candidate, but do not over-describe the failed image unless using it as an image reference. Focus on the corrected desired result.
- Move failed requirements higher in the prompt. If the IV placement failed, put the IV constraint in the core scene and medical constraints, not only in negative prompts.
- Reduce competing details. If the clinical object is wrong, simplify background and secondary actions.
- Use a tighter camera. Close-ups often improve device and anatomy accuracy; wide shots improve scene geography but reduce detail accuracy.
- Keep style language stable. Do not change visual style while debugging a medical problem unless style caused the problem.

14. Quality Control Rubric

Every candidate panel should be scored before acceptance. A beautiful image can still fail if it contradicts the panel description, breaks continuity, or shows incorrect medicine. Use a structured rubric and require remediation for any critical failure.

Domain	3 = Pass	2 = Minor issue	1 = Major issue	0 = Critical failure
Source fidelity	All required story elements visible and correctly prioritized.	One minor detail missing but story intact.	Important detail missing or altered.	Wrong scene or story beat.
Character continuity	All recurring characters recognizable and in correct state.	Small wardrobe/material drift.	Identity or state partly wrong.	Character is unrecognizable or wrong person.
Prop/device continuity	Props/devices stable and correctly placed.	Minor scale/color issue.	Important object state wrong.	Medically important device missing/wrong.
Scene continuity	Room geography, lighting, and time consistent.	Small background drift.	Room layout or direction noticeably changes.	Scene contradicts prior panels.
Medical accuracy	Clinically correct, safe, respectful, no misleading details.	Minor noncritical simplification.	Potentially misleading detail.	Unsafe, incorrect, or clinically false depiction.
Style compliance	Felt characters and 3D animated world match style bible.	Small material inconsistency.	Style partially wrong.	Wrong visual style or copied IP feel.
Slide usability	Clear focal point, safe space, no clutter, works at presentation size.	Slight crop/space issue.	Hard to read or compose with text.	Unusable as slide image.
Text hygiene	No unintended text; any text is exact and approved.	Tiny unreadable decorative marks.	Incorrect or distracting text.	Fake clinical text/dose/value.

Acceptance rule

Accept only images with no 0 scores, no medical accuracy score below 3 for externally used medical education, and no continuity score below 2. For professional or patient-facing clinical use, require human medical review before final approval. Any fake clinical text, wrong device placement, invented medication dose, or unsafe procedure depiction is a critical failure.

Deck-wide final checks

- Flip through the deck in order and track each recurring entity. Note every unexpected change in face, clothing, prop, device, injury, room layout, time, lighting, or emotional state.
- Run a separate medical pass looking only for anatomy, PPE, devices, patient positioning, medication text, monitor values, procedure safety, and stigmatizing depiction.
- Run a slide-design pass looking only for crop, text contrast, title placement, image resolution, gutters, margins, and reading order.
- Run a narrative pass looking only for whether each slide advances the story and whether transitions are understandable.
- Record all edits in a version log with prompt version, image version, reviewer, issue, action, and final status.

15. Medical Accuracy Red Flags

The AI should pause and request or apply medical review when any red flag appears. Red flags do not always mean the image cannot be generated; they mean the prompt must be clinically controlled and the output should be reviewed carefully.

Red flag	Why it matters	Prompt/QC response
Medication dose, route, or label appears	Incorrect doses or routes can be dangerous.	Use exact approved text or move to slide layer; forbid invented labels.
Procedure step depicted	Small visual errors can teach unsafe technique.	Specify sequence, PPE, sterile field, sharps safety; SME review.
Pediatric, pregnancy, elderly, disability, or mental health content	Higher risk of stigmatizing or inaccurate representation.	Use respectful tone, correct proportions/devices, avoid stereotypes.
Anatomical cutaway or pathology	Stylization can distort clinical meaning.	Use correct orientation and simplified educational model; avoid gore.
Emergency care	Dramatic images often invent unsafe chaos.	Show controlled team roles, appropriate equipment, privacy when possible.
Infectious disease/PPE	Wrong PPE can misinform viewers.	Specify exact PPE level if clinically required; no casual mask misuse.
Monitor, lab, imaging data	Random values can be misleading.	Omit values or use approved values; add exact data in deck layer.

Red flag	Why it matters	Prompt/QC response
Surgical or invasive device	Incorrect placement is common and high stakes.	Use reference-approved constraints and human review.

Patient dignity standards

- Keep bodies covered except where exposure is clinically necessary and explicitly described.
- Use privacy curtains, blankets, gowns, and respectful camera angles in clinical scenes.
- Avoid facial expressions that blame, mock, infantilize, or sensationalize illness.
- Represent diverse patients and clinicians naturally, without making identity traits the subject unless the story requires it.
- When depicting pain, fear, disability, or chronic disease, balance realism with empathy and agency.

16. Worked Prompt Example

The example below demonstrates how a panel description becomes a panel card, continuity anchors, medical constraints, and final prompt. It is intentionally modest because most real errors occur in ordinary scenes: a device placed incorrectly, a character drifting, or a room changing between panels.

Source panel description

Source text
Panel S04-P03: Maya sits on the clinic exam table, still holding her blue rescue inhaler. Dr. Lenox kneels slightly so they are at eye level and points to a simple diagram explaining how the inhaler opens the airways. Maya looks reassured but still tired. The room should feel warm and safe.

Structured interpretation

Record	Extracted content
story beat	Clinician explains inhaler mechanism empathetically; patient begins to feel reassured.
characters	CH_MAYA, CH_DR_LENEX.
props/devices	OBJ_BLUE_INHALER, OBJ_AIRWAY_DIAGRAM.
setting	SC_PED_CLINIC_ROOM_A.
action	Maya seated, holding inhaler; Dr. Lenox kneels/leans to eye level and points to diagram.
medical constraints	Inhaler shape/orientation recognizable; diagram shows simplified bronchodilation concept without misleading anatomy; no dosing text.
emotion	Maya reassured but tired; physician calm and encouraging.
composition	Medium shot, patient foreground left, clinician midground right, diagram between them, warm safe room.

Final generation prompt

Example final prompt
Create a slide-ready comic panel image for S04-P03 in 16:9 landscape, full-bleed with clean negative space in the upper left for a later caption. Do not include speech balloons, captions, labels, medication text, or random writing inside the image. Core scene: Maya sits on the clinic exam table holding her established bright blue rescue inhaler while Dr. Lenox kneels slightly at eye level and points to a simple airway diagram, creating a warm safe teaching moment. Continuity anchors: CH_MAYA is the same handmade felt child character from prior panels: soft medium-brown felt face, short black yarn curls, yellow cardigan over a pale shirt, tired but reassured expression, small blue L-shaped inhaler in her right felt hand. CH_DR_LENEX is the same calm felt clinician: warm rectangular felt face, round tortoiseshell glasses, teal scrub top with single chest pocket, silver stethoscope with black tubing, gentle half-smile. Maintain the same warm pediatric clinic room: pale blue wall, white exam table on screen left, rounded wooden chair on screen right, small heart poster in background, soft morning window light from screen left. Composition: medium shot from patient side, Maya foreground left seated on exam table, Dr. Lenox midground right at eye level, simple airway diagram on a tablet or clipboard between them as focal support, monitor-free uncluttered background. Medical accuracy constraints: The inhaler should be recognizable as a metered-dose rescue inhaler but brand-free, with mouthpiece orientation plausible and no medication label or dose. The airway diagram should be a simplified educational illustration of airways opening, not a detailed diagnostic image, with no incorrect labels. Respectful clinical tone, patient dignity preserved. Visual style: Original premium 3D animated family-film look; human characters are handmade felt-like figures with soft visible fibers, subtle seams, plush expressive faces, rounded eyes, warm cinematic lighting; environment and medical props are clean stylized 3D animated objects, medically recognizable; emotionally warm, not silly, no existing studio characters or franchise designs. Required details: blue inhaler in Maya's right hand, Dr. Lenox pointing to diagram, eye-level posture, Maya reassured but tired, warm safe clinic atmosphere. Final output requirements: crisp focus on Maya, Dr. Lenox, inhaler, and diagram; no logos, no unintended text, no random numbers, no extra people, no anatomical gore, no incorrect inhaler use.

QC checklist for the example

- Does Maya match the established felt child design and yellow cardigan?

- Is the inhaler blue, in her right hand, and recognizable without fake labeling?
- Is Dr. Lenox at eye level rather than towering over the patient?
- Does the diagram suggest airways opening without incorrect anatomy or text?
- Does the room match the established pediatric clinic layout?
- Does the tone feel warm and respectful rather than comedic?

17. Automation and API-Oriented Practices

For automated pipelines using gpt-image-2, store prompts, source records, references, and outputs as versioned artifacts. A reliable system treats each image generation as a reproducible production event even though exact visual randomness may vary. The goal is not deterministic pixels; it is deterministic requirements and review.

Recommended data objects

Object	Contents	Used by
project_brief.json	Audience, aspect ratio, deck format, style bible, medical review level, slide text policy.	Prompt compiler and deck assembler.
panel_cards.json	Structured card for every panel/slide.	Prompt compiler.
continuity_bible.json	Character, prop, device, scene, timeline, and state records.	Prompt compiler and QC.
medical_truth_ledger.json	Clinical fact records, approved constraints, review status, red flags.	Prompt compiler and medical reviewer.
prompt_versions.jsonl	Every generation/edit prompt with timestamp, panel ID, assets used, and settings.	Audit and debugging.
image_manifest.json	Candidate and approved images, version, score, reviewer notes, slide placement.	Deck assembler.
deck_manifest.json	Slide order, image file, text overlays, alt text, notes, approval status.	Final deck build.

Reference image strategy

- Generate and approve character reference sheets before story panels. Use them as inputs for panels featuring recurring characters.
- Generate and approve key prop/device references when a prop is clinically meaningful or appears repeatedly.
- For adjacent panels in the same scene, use the previous approved panel as a visual reference only if it does not lock in a medical mistake.
- Never use an unapproved render as continuity reference for future panels; errors will propagate.
- Store reference assets with stable IDs and usage notes: “approved for identity only,” “approved for room layout,” or “approved for device placement.”

Batch generation cautions

- Do not batch-generate a whole medical deck without intermediate review. Early continuity or clinical errors will multiply.
- Batch only low-risk panels after character/style references are locked and the scene has no clinical complexity.
- Use a two-pass approach: draft candidates quickly, then regenerate high-priority slides at higher fidelity after QC.
- Maintain a failure taxonomy so recurrent issues become prompt-library improvements rather than one-off fixes.

18. Prompt Library: Reusable Modules

Reusable modules make prompts shorter and more consistent. Store them as named snippets and insert only the snippets relevant to each panel.

Module name	Snippet
STYLE_GLOBAL_FELT_3D	Original premium 3D animated family-film visual language; human characters are handmade felt-like figures with visible fibers, subtle seams, plush expressive faces, rounded eyes, warm cinematic lighting; environments and medical equipment are clean stylized 3D animated objects; respectful clinical tone; no existing studio characters or franchise designs.
TEXT_FREE_MEDICAL	Do not include speech balloons, captions, labels, medication text, monitor values, brand logos, or random writing inside the image. Any exact text will be added in the slide deck.
PATIENT_DIGNITY	Preserve patient dignity: respectful camera angle, appropriate gown/blanket coverage, calm empathetic body language, no sensationalized pain or embarrassment.
DEVICE_ACCURACY	Medical devices must be stylized but structurally recognizable, correctly placed, logically connected, brand-free, and free of invented values or labels.
ANATOMY_INSET	If showing internal anatomy, render it as a clearly educational non-graphic inset or model with correct orientation and simplified anatomy; do not merge literal gore with the patient scene.
SAFE_PROCEDURE	Show clinically safe procedure behavior: correct PPE if applicable, clean/sterile field as appropriate, safe sharps handling, plausible patient positioning, and no skipped safety-critical step.
SLIDE_SAFE_SPACE	Create a clean slide-ready composition with uncluttered negative space in [location] for later deck text; keep focal clinical/story element unobstructed.
NO_DRIFT	Maintain established continuity of character identity, felt material, wardrobe, prop color, device placement, scene layout, lighting direction, and current emotional/clinical state.

Common negative constraints

- No random or invented text, numbers, medication labels, lab values, monitor readings, brand logos, or watermarks.
- No extra people, extra limbs, distorted hands, impossible device connections, or floating props.
- No incorrect laterality; use patient-perspective left/right when anatomy matters.
- No unsafe clinical technique, contaminated sterile field, exposed needle left in vein, disconnected oxygen tubing, or nonsensical monitor leads.
- No gore unless explicitly required and medically reviewed; prefer non-graphic educational stylization.
- No copying existing animated characters, studio logos, trademarked costumes, or proprietary scene designs.

19. Troubleshooting Failure Modes

When failures repeat, the problem is usually upstream: weak panel parsing, incomplete continuity anchors, vague medical requirements, or an overloaded prompt. Fix the system, not only the single image.

Failure mode	Diagnosis	System-level correction
Images look pretty but ignore the panel	Prompt foregrounds style more than required visuals.	Move required visuals above style; use source fidelity checklist.
Characters drift across slides	No reference sheet or stable anchor phrase.	Create character sheets; use concise identity anchors in every prompt.
Objects change color/shape	Props not entered into continuity bible.	Create prop records and anchor phrases; place prop in required visuals.
Medical devices look decorative	Device accuracy not specified.	Add device type, placement, connection, and forbidden errors.
Scene geography flips	No scene map or camera axis.	Add room layout anchor and camera side instruction.
Too much generated text	Prompt did not prohibit text.	Add TEXT_FREE_MEDICAL module and use deck text layer.
Clinical content is inaccurate	Medical truth ledger missing or vague.	Create ledger, simplify composition, regenerate with exact visible constraints.
Style becomes clay/plastic	Felt language not reinforced.	Repeat felt material details for every human character.
Images are too childish	Felt style interpreted as toy comedy.	Add respectful clinical tone, patient dignity, realistic device material.

Failure mode	Diagnosis	System-level correction
Slide text has no space	Prompt not slide-aware.	Add safe-space directive and regenerate with composition controls.

20. Appendices: Copy-Ready Checklists and Schemas

Appendix A: Project kickoff checklist

- Audience defined: patient, caregiver, trainee, clinician, public health, internal storyboard, marketing, or entertainment.
- Medical review level defined: no clinical content, low-risk education, moderate-risk clinical depiction, high-risk procedure/medication/emergency.
- Deck specs defined: aspect ratio, slide size, number of panels per slide, text overlay policy, export format.
- Style bible approved: felt characters, 3D animated environment, tone, lighting, color palette, no copied IP.
- Source document parsed into panel cards with no unreviewed ambiguous medical details.
- Continuity bible created for characters, props, devices, locations, and timeline states.
- Medical truth ledger created and reviewed for all clinically meaningful panels.
- Reference sheets generated and approved for recurring characters and critical props/devices.
- Prompt templates and negative constraints selected.
- QC rubric and acceptance thresholds agreed before generation starts.

Appendix B: Medical reviewer checklist

- Anatomy: correct body region, laterality, scale, orientation, non-graphic stylization, and no misleading distortion.
- Devices: correct type, placement, connection, size, display, tubing/leads, and use state.
- Procedures: correct sequence, PPE, sterile/clean technique, sharps safety, patient positioning, and privacy.
- Medication: no invented dose, route, label, package, prescription details, or administration implication.
- Clinical environment: appropriate care setting, hygiene, equipment, staff roles, and patient coverage.
- Patient representation: dignity, empathy, diversity, non-stigmatizing posture/expression, appropriate age/proportions.
- Educational clarity: visual metaphor is clearly symbolic if nonliteral; labels/text are exact and approved if present.
- Risk communication: image does not exaggerate, minimize, or sensationalize risk beyond the script.

Appendix C: Continuity review checklist

- Character face, felt material, hair/head shape, clothing, accessories, size relationship, and emotional state remain consistent.
- Props and devices keep correct color, shape, location, owner, use state, and clinical logic.
- Scene layout remains stable: doors, windows, bed/table/chair, monitors, posters, lighting direction, camera side.
- Timeline is coherent: injuries, bandages, lines, fatigue, weather, time of day, and object positions change only when motivated.
- Medical progression is coherent: interventions appear in sequence and do not vanish unless removed in the story.
- Style remains consistent: felt characters do not become plastic, realistic humans, clay models, or flat illustrations.

Appendix D: JSON-like schema for implementation

Implementation schema
<pre>{ "project_brief": { "deck_aspect_ratio": "16:9", "visual_style": "STYLE_GLOBAL_FELT_3D", "text_policy": "exact educational text added in slide editor", "medical_review_level": "moderate/high", "audience": "patient education / clinician training / other" }, "panel_card": { "panel_id": "S##_P##", "source_text": "verbatim description", "story_beat": "one-sentence function", "visible_moment": "single frozen instant", "characters_present": ["CH_ID"], "settings": ["SC_ID"], "props_devices": ["OBJ_ID", "DEV_ID"], "required_visuals": [], "camera": {"shot": "medium", "angle": "patient-side", "safe_space": "upper left"}, "medical_ledger_ids": ["MED_ID"], "continuity_state_updates": [], "text_layer": "none in image; caption in deck", "risk_flags": [] } }</pre>


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  "immutable_anchors": [],
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  "clinical_fact": "approved fact",
  "visible_requirement": "what image must show",
  "must_not_show": [],
  "review_status": "pending / approved / rejected",
  "reviewer_notes": ""
}
}
```

Appendix E: Final deck sign-off form

Sign-off area	Reviewer	Status	Notes
Source fidelity		Pending / Approved / Needs revision	
Character continuity		Pending / Approved / Needs revision	
Prop/device continuity		Pending / Approved / Needs revision	
Medical accuracy		Pending / Approved / Needs revision	
Patient dignity and tone		Pending / Approved / Needs revision	
Slide design and accessibility		Pending / Approved / Needs revision	
Text/caption proofing		Pending / Approved / Needs revision	
Export QA		Pending / Approved / Needs revision	

References Used for OpenAI Product Terminology

These references were used only to confirm current OpenAI image-generation naming and workflow capabilities. The production, continuity, and medical-review practices in this guide are operational recommendations built around those capabilities.

- OpenAI. "Introducing ChatGPT Images 2.0." April 21, 2026. <https://openai.com/index/introducing-chatgpt-images-2-0/>
- OpenAI Help Center. "ChatGPT Release Notes." April 21, 2026 entry: ChatGPT Images 2.0 in ChatGPT. <https://help.openai.com/en/articles/6825453-chatgpt-release-notes>
- OpenAI Help Center. "Images in ChatGPT." <https://help.openai.com/en/articles/11084440-images-in-chatgpt>
- OpenAI Developers. "Image generation | OpenAI API." <https://developers.openai.com/api/docs/guides/image-generation>
- OpenAI Developers Cookbook. "GPT Image Generation Models Prompting Guide." <https://developers.openai.com/cookbook/examples/multimodal/image-gen-models-prompting-guide>

Final production reminder

Ship only after a full pass

The final deck should be reviewed in order, at presentation size, by someone checking story continuity and by an appropriate medical reviewer for clinically meaningful content. Keep the final prompt, image version, medical ledger IDs, and approval status with every slide so revisions remain traceable.